

SYNTHETIC

H-Cys-Tyr-Phe-Glu (NH₂)-Asp (NH₂)-Cys-Pro-Arg-Gly-NH₂
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Page 1 of 4

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At infusion rates used in vasodilatory shock (0.01 – 0.1 units/minute), the clearance of vasopressin is 9 to 25 ml/min/kg in patients with vasodilatory shock. The apparent $t_{1/2}$ of vasopressin at these levels is ≤ 10 minutes. Vasopressin is predominantly metabolised and only about 6% of the dose is excreted unchanged in urine. Animal experiments suggest that the metabolism of vasopressin is primarily by liver and kidney. Serine protease, carboxipeptidase and disulfate oxido-reductase cleave vasopressin at sites relevant for the pharmacological activity of the hormone. Thus, the generated metabolites are not expected to retain important pharmacological activity.

INDICATIONS AND CLINICAL USE

Vasopressin Kabi Injection, USP is indicated for prevention and treatment of postoperative abdominal distention, in abdominal roentgenography to dispel interfering gas shadows, and in diabetes insipidus.

CONTRAINDICATIONS

Vasopressin Kabi Injection, USP is contraindicated in patients with known allergy or hypersensitivity to 8-L-arginine or chlorobutanol.

WARNINGS AND PRECAUTIONS

Worsening Cardiac Function.

Use in patients with impaired cardiac response may worsen cardiac output.

INTERACTIONS WITH OTHER MEDICAMENTS

Catecholamines

Use with catecholamines is expected to result in an additive effect on mean arterial blood pressure and other hemodynamic parameters.

Indomethacin

Use with indomethacin may prolong the effect of Vasopressin Kabi on cardiac index and systemic vascular resistance

Ganglionic Blocking Agents

Use with ganglionic blocking agents may increase the effect of Vasopressin Kabi on mean arterial blood pressure.

Furosemide

Use with furosemide increases the effect of Vasopressin Kabi on osmolar clearance and urine flow

Drug Suspected of Causing SIADH

Use with drugs suspected of causing SIADH (e.g. SSRIs, tricyclic antidepressants, haloperidol, chlorpropamide, enalapril, methyldopa, pentamidine, vincristine, cyclophosphamide, ifosfamide, felbamate) may increase the pressor effect in addition to the antidiuretic effect of Vasopressin Kabi.

Drug Suspected of Causing Diabetes Insipidus

Use with drugs suspected of causing diabetes insipidus (e.g. demeclocycline, lithium, forcarnet, clozapine) may decrease the pressor effect in addition to the antidiuretic effect of Vasopressin Kabi.

PREGNANCY AND LACTATION

Pregnancy Category C

Risk Summary: There are no adequate or well-controlled studies of Vasopressin Kabi Injection, USP in pregnant women. It is not known whether vasopressin can cause fetal harm when administered to a pregnant woman or can affect reproduction capacity. Animal reproduction studies have not been conducted in vasopressin.

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Clinical Considerations: Because of increased clearance of vasopressin in the second and third trimester, the dose of Vasopressin Kabi may need to be up-titrated to doses exceeding 0.1 units/minutes in post-cardiotomy shock and 0.07 units/minute in septic shock.

Vasopressin Kabi Injection, USP may produce tonic uterine contractions that could threaten the continuation of pregnancy.

Lactation

It is not known whether vasopressin is present in human milk. However, oral absorption by nursing infant is unlikely because vasopressin is rapidly destroyed in gastrointestinal tract. Consider advising a lactating woman to pump and discard breast milk for 1.5 hours after receiving vasopressin to minimize potential exposure to the breastfed infant.

ADVERSE REACTIONS

The following adverse reactions associated with the use of vasopressin were identified in literature. Because these reactions are reported voluntarily from the population of uncertain size, it is not possible to estimate the frequency reliably or to establish a causal relationship to drug exposure.

Bleeding/lymphatic system disorders: Haemorrhagic shock, decreased platelets, intractable bleeding

Cardiac disorders: Right heart failure, atrial fibrillation, bradycardia, myocardial ischemia.

Hepatobiliary: Increased bilirubin levels.

Renal/urinary disorders: Acute renal insufficiency

Vascular disorders: Distal limb ischemia

Metabolic: Hyponatremia

Skin: Ischemic lesions

DOSAGE AND ADMINISTRATION

Vasopressin Kabi Injection, USP may be administered intramuscularly (i.m.) or subcutaneously (s.c.).

Ten units (0.5 mL) of Vasopressin Injection will usually elicit full physiologic response in adult patients; 5 units (0.25 mL) will be adequate in many cases. Vasopressin Injection should be given i.m. at three or four hour intervals as needed. The dosage should be proportionately reduced for children. (For an additional discussion of dosage, consult the sections below.)

When determining the dose of Vasopressin Kabi Injection, USP for a given case, the following should be kept in mind:

It is particularly desirable to give a dose not much larger than is just sufficient to elicit the desired physiologic response. Excessive doses may cause undesirable side actions – blanching of the skin, abdominal cramps, nausea – which, though not serious, may be alarming to the patient. Spontaneous recovery from such side actions occurs in a few minutes. It has been found that one or two glasses of water given at the time Vasopressin Kabi Injection, USP is administered reduce such symptoms.

Abdominal Distention

In the average postoperative adult patient, give 5 units (0.25 mL) initially, increase to 10 units (0.5 mL) at subsequent injections if necessary. It is recommended that vasopressin be given i.m. and that injections be repeated at three or four hour intervals as required. Dosage to be reduced proportionately for children.

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Vasopressin used in this manner will frequently prevent, or relieve, postoperative distention. These recommendations apply also to distention complicating pneumonia or other acute toxemias.

Abdominal Roentgenography

For the average case, two injections of 10 units (0.5 mL) each are suggested. These should be given two hours and one-half hour, respectively before films are exposed. Many roentgenologists advise giving an enema prior to the first dose of vasopressin.

Diabetes Insipidus

Vasopressin may be given by injection or administered intranasally on cotton pledgets, by nasal spray, or by dropper. The dose by injection is 5 to 10 units (0.25 to 0.5 mL) repeated two or three times daily as needed. When Vasopressin Injection is administered intranasally by spray or on pledgets, the dosage and interval between treatments must be determined for each patient.

OVERDOSE AND TREATMENT

Overdosage with Vasopressin Kabi Injection, USP can be expected to manifest as consequences of vasoconstriction of various vascular beds (peripheral, mesenteric and coronary) and as hyponatremia. In addition, overdosage may lead less commonly to ventricular tachyarrhythmias (including Torsade de Pointes), rhabdomyolysis, and non-specific gastrointestinal symptoms.

Direct effects will resolved within minutes of withdrawal of treatment

INCOMPATIBILITIES

To avoid incompatibilities, Vasopressin Kabi Injection, USP should not be mixed with other drugs.

Parenteral drug products should be inspected visually for particulate matter and discoloration prior to administration, whenever solution and container permit.

AVAILABILITY OF DOSAGE FORMS

Vasopressin Kabi Injection, USP 20 Units/mL, 1 mL in 2 mL (partially filled) flip-top vials in packages of 25.

STORAGE AND STABILITY

Store below 25°C. Do not permit to freeze.

MANUFACTURER



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